

Eating and Swallowing Problems in People With Advanced Dementia

Patients with advanced dementia commonly have problems with chewing and swallowing, which may lead to aspiration of food, weight loss, and malnutrition.

Clinicians should ask caregivers if patients with advanced dementia have difficulties with chewing and swallowing and whether they need assistance with eating. Speech-language pathologists can observe swallowing function and recommend strategies to feed patients in the safest manner possible.

What Is Hand Feeding?

Hand feeding (also called assisted oral feeding) involves caregivers providing food and drink to patients using strategies that decrease aspiration, including low-flow cups to limit large volumes of liquids, feeding patients when they are alert, and ensuring upright positioning. Hand feeding with small amounts of food and fluids can help avoid feelings of hunger and thirst. Although hand feeding is time-consuming, it may enhance quality of life through social interaction and allow people with advanced dementia the continued pleasure of tasting food.

What Are Feeding Tubes?

Feeding tubes are plastic tubes that can be inserted into the gastrointestinal tract to provide nutrition, fluids, and medications for people who are unable to eat food by mouth. Long-term feeding tubes are surgically inserted through the abdominal wall into the stomach or small intestine using an endoscope to help guide placement.

Feeding tubes do not improve nutrition, quality of life, or survival and do not prevent aspiration pneumonia or improve pressure ulcer healing in patients with advanced dementia.¹ Among these patients, feeding tubes, which are often inserted during hospitalization for an acute illness, are associated with increased admissions to the intensive care unit, longer hospital stays, higher rehospitalization rates, and higher mortality compared with patients with advanced dementia who do not have long-term feeding tubes.

Potential Side Effects of Feeding Tubes

Some patients may have pain, infection, and bleeding at the feeding tube insertion site. The feeding tube may also fall out or migrate outside of the stomach or small intestine. If patients with dementia attempt to pull out feeding tubes, restraints are sometimes used, which may result in decreased mobility, deconditioning, incontinence, pressure ulcers, and behavioral disturbances. Feeding tubes also deprive patients of the pleasure of eating.

Eating and swallowing problems are common in patients with advanced dementia and can lead to malnutrition, weight loss, and aspiration of food.

 Speech-language pathologists can observe swallowing function and recommend feeding strategies to caregivers and patients.

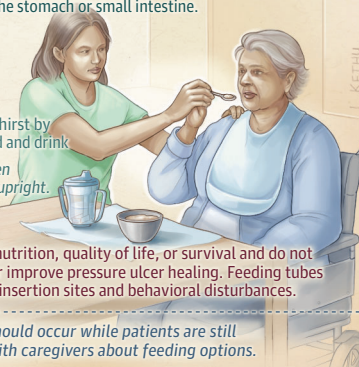
Hand feeding has more benefits and fewer risks than feeding tubes that are surgically inserted into the stomach or small intestine.

- ☒ Allows patient the pleasure of tasting food
- ☒ Enhances quality of life through social interaction
- ☒ Helps manage hunger and thirst by giving small amounts of food and drink

Hand feeding should be done when patients are alert and positioned upright.

Feeding tubes do not improve nutrition, quality of life, or survival and do not prevent aspiration pneumonia or improve pressure ulcer healing. Feeding tubes may cause pain and infection at insertion sites and behavioral disturbances.

 Advance care planning should occur while patients are still able to make decisions with caregivers about feeding options.



Recommended Feeding Options for Patients With Advanced Dementia

Guidelines for patients with advanced dementia recommend careful hand feeding in most cases. Geriatric or palliative care teams can discuss feeding options with caregivers and explain that weight loss and lower calorie intake are expected as dementia progresses. Attempts at hand feeding should stop if patients experience choking or gagging with assisted oral feeding. Use of moist mouth swabs may help decrease dry mouth and enhance swallowing.

Advance Care Planning

Advance care planning and education about feeding options should be provided before patients develop advanced dementia and cannot make their own decisions. Patients with dementia and their families should be educated about the benefits of hand feeding and the possible risks and complications associated with feeding tubes.

FOR MORE INFORMATION

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